



Recognition and Response to Acute Deterioration Policy

1. Applicability

Site	Operational Area	Applicable to
Armadale Kalamunda Group	All areas	All workforce groups

2. Purpose

To outline the escalation protocols in response to recognition of deterioration for staff, patient, family and carers. To ensure safe, early and appropriate management of deteriorating patients and medical emergencies occurs across Armadale Kalamunda Group (AKG).

3. Scope

This policy is applicable to all adult, paediatric and neonate patient cohorts within the Armadale Kalamunda Group.

4. Assessment requirements

4.1 Physiological observations

Physiological observations should be documented and monitored on an [approved Observation and Response Chart](#).

At a minimum a full set of physiological observations will be documented:

- At the time of presentation or admission
- A frequency determined by the patient's clinical status and/or current treatment
 - At least every eight hours for an acute admission
 - Or for at least every 24 hours for sub-acute episodes
- Prior to inter or intra-hospital transfer
- When a patient is experiencing, or at risk of experiencing an episode of acute deterioration.

4.2 Mental state assessment

The mental health assessment should be documented and monitored in the healthcare record.

At a minimum a mental state assessment will be documented:

- Time of presentation or admission
- Any changes in behaviour, cognitive function, perception and/or emotional state

The following should be considered in the assessment:

- Delirium (perform 4AT)
- Cognitive impairment

- Distress
- Loss of touch with reality or consequence of behaviours
- Loss of function
- Elevated risk to self, others or property
- A pre-existing mental health diagnosis
- Any treatment for a mental health condition
- Any patient, family, carer or NOK concerns regarding mental state changes in the past seven (7) days (perform a Single Question in Delirium (SQUID))

Refer to the [Mental State Deterioration and Escalation Pathway](#)

5. Escalation protocols

5.1 Physiological deterioration

- Escalation protocols are determined by scores recorded on the Observation and Response Charts.
- Initiate sepsis screening for a patient >14 years with features of infection, in whom the diagnosis is uncertain, or who looks unwell and commence Sepsis Pathway.

Non-admitted patient services (excluding Emergency Department presentations):

- Where non-urgent physiological deterioration is detected during a non-admitted patient interaction (e.g., outpatient clinic / community health service) the matter is to be escalated to the treating team for consideration of further actions.
- Any escalation and action(s) taken to address physiological deterioration in the non-admitted patient setting are to be documented in the patient's healthcare record.
- For physiological deterioration requiring emergency medical attention a [Code Blue-Medical Emergency](#) is to be activated.

5.2 Mental state deterioration (any new confusion or altered mental state)

If a patient has a 4AT of greater than 3 or Single Question in Delirium (SQiD) positive or any other triggers on the mental state assessment:

- Commence a Behavioural Observation Form (BOF)
- Inform the Nurse/Midwife Shift Coordinator and the Medical Officer
- Consider formal delirium and/or cognitive assessment – refer to the [Delirium Clinical Management Guideline](#)
- Consider referral to the Mental Health Consultation Liaison Service:
 - Between 08:30am to 04.30pm via phone on 0418 948 296 or 0408 677 504

Refer to the [Mental State Deterioration and Escalation Pathway](#)

5.3 Special and supportive observations

- When a patient is placed on forms under the MHA (2014) a 1:1 special (mental health where available) is to be provided.
 - Discuss with treating psychiatric team to determine if a suitable alternative can be allocated based on risks, care requirements and observations.
 - Document outcome of discussion in patient integrated notes.

6. Modifications

If abnormal observations are to be tolerated for individual patient circumstances (such as Advanced Health Directives and Goals of Patient Care), a modification must be documented on the Observation and Response Chart authorised by a registrar or consultant.

6.1 Goals of Patient Care

- The GoPC eForm should be completed within 48 hours of admission (where appropriate) – see the [Goals of Patient Care eForm Procedure](#) for further detail.

If there is no documented evidence of a completed GoPC form:

- The default is for full cardiopulmonary resuscitation (CPR) to occur
- If a patient is thought to be unlikely to survive CPR by senior clinicians, then the treatment does not have to be offered on the grounds of harm rather than benefit.

7. Rapid Response Required

Activate a Code Blue / Medical Emergency Team (MET); Neonatal; Obstetric; Caesarean Section; Adult; Paediatric:

- If a patient's condition meets the [criteria](#) for a Code Blue / MET
- An urgent medical review has been requested and not attended within 10 minutes
- Staff concerns of patient deterioration that does not meet MET criteria
- Patient, family/carer concerns
- Multidisciplinary post MET call huddle for medical management and monitoring plan for all patients following a MET call including:
 - Initiate appropriate interventions
 - Formulate a management plan, including location and level of care

8. Communication

8.1 Clinical handover

Clinical handover process must adhere to the [Clinical Handover Procedure](#) requirements using the ISoBAR framework and include:

- Patients of concern
- Patients who are at risk of deterioration
- The most recent set of physiological observations
- The monitoring plan including any changes/modifications made to the frequency and type of observations required.

8.2 Communication with the patient, family and/or carer

Communication with patients, family, carers and significant others should include:

- Education regarding patient, family and carer initiated escalation of care processes – refer to the [Aishwarya's CARE Call Procedure](#).
- Discussion for patient centered information to inform assessment and interventions such as [Top '5](#).

9. Compliance monitoring

Compliance against this policy will be monitored by the Deteriorating Patient and Resuscitation Committee.

Compliance monitoring activities relevant to this policy include:

- Monthly monitoring of:
 - Category 1 caesarean section decision to incision interval > 30mins
- Bi-annual compliance auditing of:
 - Fetal Surveillance and Cardiotocography (CTG) Monitoring Policy
- Quarterly auditing of MET CALLS:
 - ADDS score aligns with escalation pathway (actions/interventions)
 - GoPC completed
- Auditing and monitoring of Observation and Response Chart:
 - Physiological observations are recorded
 - Care escalation according to protocol
- Monitoring of MySay survey results for:
 - Awareness of the Aishwarya CARE
 - Patient confidence to ask for help
- Monthly monitoring of:
 - Reported clinical incidents in Datix CIMS related to patient deterioration
 - Sepsis Management Compliance
- Quarterly auditing of code black incidents related to mental state deterioration:
 - Behavioural Observation Form or Safe and Supportive Observation Chart
 - Assessment aligns with the escalation pathway

10. Legislative context

The following documents are required to give effect to this policy:

- HDWA [Recognising and Responding to Acute Deterioration Policy MP0086/18](#)

11. Supporting information

The following documents support the implementation of this policy:

- EMHS [Sepsis Management Policy](#)
- AKG [Clinical Handover Procedure](#)
- AKG [Aishwarya's CARE Call Procedure](#)
- AKG [Goals of Patient Care eForm Procedure](#)
- AKG [Mental Health Consultation Liaison Service Procedure](#)
- AKG [Advance Health Directive and Enduring Power of Guardianship Guideline](#)
- AKG [Neonatal Routine and Additional Observations Guideline](#)
- AKG [Delirium Clinical Management Guideline](#)

12. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2
- Standard 5
- Standard 6
- Standard 8

13. References

1. Australian Commission on Safety and Quality in Health Care (2010). [National Consensus Statement: Essential Elements for Recognising and Responding to Clinical Deterioration](#). Sydney, ACSQHC.
2. Australian Commission on Safety and Quality in Health Care. [National Consensus Statement: Essential elements for recognising and responding to deterioration in a person's mental state](#). Sydney: ACSQHC; 2017
3. Australian Commission on Safety and Quality in Health Care. [National Safety and Quality Health Service Standards User guide for health service organisations providing care for patients with cognitive impairment or at risk of delirium](#). Sydney: ACSQHC; 2019.
4. Australian Commission on Safety and Quality in Health Care. [Delirium Clinical Care Standard](#). Sydney: ACSQHC, 2016.

14. Review

This policy will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V4.1	12/04/2024	12/04/2026	Minor amendment to compliance monitoring

15. Authorisation

This policy has been authorised and issued by Executive Director:

Authorised by	Neil Cowan – Executive Director
Authorisation date	11/04/2024
Published date	12/04/2024

Appendix 1 – Mental State Deterioration and Escalation Pathway

